

--

Name: امينه محمد ابو زيد عزوز National ID: 27609230103425	
--	---

تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بان نوضح فيما يلي تقريرنا عن حالته
--

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
---	---

وبعد الكشف والفحوصات والمتابعة وجد انه
--

المریضة تعاني من ورم غير حميد بالثدى و تحتاج الي Supportive and palliative Treatment
--

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين

٢٠٢٦/٠١ ٢٧٦.٩٢٣.٠١.٣٤٢٥

ربة منزل

أنثى مسلمة متزوجة

محمد عبد المنعم عبد الحميد محمد

البطاقة سارية حتى ٢٠٢٣/٠١/٢١ ١



جمهورية مصر العربية

بطاقة تحقيق الشخصية

امينه

محمد ابوزيد عزور عيسى

١٥ ش احمد يونس - بشتيل البلد

مركز اوسيم - الجيزة

١٩٧٦/٠٩/١٤

٢٧٦.٩٢٣.٠١.٣٤٢٥

LP0385235



Patient Name : الأستاذ / امينه محمد ابو زيد عزوز
Patient No. : 27609230103425
Path No. : 1473/26
Sex : Female
Age : 49 Y
Referring Doc. :

Request No. : 13006486
Request Date : 12/05/2026
Verification Date : 07/06/2026
Financial Category : جوستاف روسي - مبادره
Department : Out Patient
IHC No. :
Registered by : .marwa mohamed youss

IMMUNOHISTOCHEMISTRY REPORT

Clinical Data: Clinical Data: Right breast locally advanced retroareolar mass "BIRADs 5" diagnosed "Invasive duct carcinoma, grade 2".

Nature of Specimen: Tru-cut biopsy, collected on 12/5/2026, at 1:15 pm, Cold ischemia time: 20 seconds.

Technique:

- Sections prepared from paraffin block for immune-staining using Bench Mark ULTRA system for;
- ER clone (1E2), PR clone (SP1), Her2 clone (4B5), & Ki67 clone (30-9).

Results:

* IPT (All controls showed appropriate reactivity):

- ER: Strong nuclear reaction in 95% of tumor cells (score 8/8).
- PR: Strong nuclear reaction in 50% of tumor cells (score 7/8).
- Her2: Faint incomplete membrane reaction in less than 10% of tumor cells (Ultra-low, score 0+).
- Ki67%: 30%.

Conclusion:

Right breast mass, Tru-cut biopsy:

IPT:

- ER: Positive (score 8/8).
- PR: Positive (score 7/8).
- Her2: Ultra low (score 0+).
- Ki67%: 30%.

* Scoring system: "ER, PR"

ASCO-CAP ER/PR Test Guideline Recommendations: proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66%)

Verified by :

Dr.Duaa Mohamed

Printing Date : 10/06/2026 11:51:06

Page 1 of 2

Printed By : .marwa mohamed youss



غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Name: _____ MR. : 11516	امينه محمد ابوزيد عزوز عيسى	
Detailed Medical Report		
		السيد
تحية طيبة وبعد,,		
	بشأن تقرير طبي للمريض	إيماء للطلب المقدم من
	رقم طبي:	بقسم:
ننشرف بأن نوضح فيما يلي تقريراً عن حالته		
	تاريخ وساعة الخروج :	تاريخ وساعة الدخول :
وبعد الكشف والفحوصات والمتابعة وجد انه		
The patient presents with invasive ductal carcinoma (IDC) that is estrogen receptor (ER) 8/8, progesterone receptor (PR) 7/8, HER2 negative, and Ki-67 30 %. Imaging demonstrates a retroareolar mass with nipple retraction and increased periareolar skin thickness, measuring 3.5 cm × 2 cm with intraductal extension to the nipple. BIRADS 5 and BIRADS 6 findings on contrast-enhanced mammography (CESM) reveal a 9.7 cm × 4.4 cm mass with skin thickening and a faint 5 cm non-mass enhancement at 1 o'clock, also extending intraductally. The multidisciplinary team has recommended a port-cath insertion for neoadjuvant chemotherapy (NACT) consisting of three cycles of AC (doxorubicin and cyclophosphamide) followed by three cycles of docetaxel, with subsequent radiotherapy (RT) and sentinel lymph node biopsy (SLNB). The second attending note indicates the patient is a stage CT3 N1 breast cancer with the same receptor profile (ER 8/8, PR 7/8, HER2-negative, Ki-67 30 %) and a 66 % response on CESM after the first cycle of AC. The plan includes continuation of neoadjuvant therapy with AC and docetaxel, followed by definitive surgery and adjuvant treatment. The patient's weight is 82 kg, height 152 cm, and body surface area 1.87 m². No explicit cycle or week count is provided in the most recent note.		
تحريراً في ,		
	الوقت :	التاريخ : 20-09-2026
Reham Ahmed Fathy Ibrahim		الطبيب المعالج :
يعتمد ,		

Patient Name : الأستاذة / امينه محمد ابو زيد عزوز :
Patient No. : 27609230103425
Path No. : 1473/26
Sex : Female
Age : 49 Y
Referring Doc :

Request No. : 13006486
Request Date : 12/05/2026
Verification Date : 07/06/2026
Financial Category : جوستاف روسي - مبادره
Department : Out Patient
Registered by : .marwa mohamed youss

Pathology Report

Clinical Data: Right breast locally advanced retroareolar mass "BIRADs 5".
Nature of Specimen: Tru-cut biopsy, collected on 12/5/2026, at 1:15 pm, Cold ischemia time: 20 seconds.

Gross Examination:

- Fragmented grey white tissue cores, ranging in size between 0.5 cm & 1.3 cm long, totally submitted.

Microscopic Examination:

- * Histological type: Invasive duct carcinoma.
- * Grade 2 (differentiation 3, nuclear pleomorphism 2, mitosis 2).
- * Ductal carcinoma in situ: Absent.
- * Perineural invasion: Absent.
- * Angioinvasion: Absent.
- * Microcalcification : Absent
- * Necrosis: Focal.
- * Tumor infiltrating lymphocytes (TILs): Mild.

Diagnosis:

Right breast mass, Tru-cut biopsy:
- Invasive duct carcinoma, Grade 2.
- For further IHC (ER, PR, Her2, Ki67).

Signature

Dr. Walaa Ibrahim Ghoneim.

Verified by :

Dr.Duaa Mohamed

Printing Date : 10/06/2026 11:49:56

Page 1 of 2

Printing By : .marwa mohamed youss



غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Patient Name الأستاذ / امينه محمد ابو زيد عزوز
Patient No. 27609230103425
Path No. 1473/26
Sex Female
Age 49 Y
Referring Doc.

Request No. 13006486
Request Date 12/05/2026
Verification Date 07/06/2026
Financial Category جوسفان روسي - مبادره
Department Out Patient

Registered by .marwa mohamed yousse

IMMUNOHISTOCHEMISTRY REPORT

and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, weak, 2, intermediate; and 3, strong). Total score from 0 to 8.

* Scoring system: "Her2"

Reference: ASCO-CAP Her2 Test Guideline Recommendations. IHC 0 (Null): as defined by no staining observed. IHC 0+ (Ultra-low): as defined by membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within > 10% of the invasive tumor cells.

IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within >10% of

the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is complete, intense > 10% of tumor cells.

Signature

Dr. Walaa Ibrahim Ghoneim.

Verified by :
Dr.Duaa Mohamed

Printing Date : 10/06/2026 11:51:06

Page 2 of 2

Printed By : .marwa mohamed yousse



غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

CAIRO UNIVERSITY HOSPITALS
FACULTY OF MEDICINE
RADIO DIAGNOSIS DEPARTMENT

مستشفيات جامعة القاهرة
كلية الطب
قسم الأشعة التشخيصية

WOMEN IMAGING DEPARTMENT

Patient Name: Amina Mohammed Abozeid

Date: 26-04-2026

Bilateral mammography with complementary High resolution Ultrasound Examination

Indication: Right breast lump.

Findings:

- Heterogeneously dense breast parenchyma (ACR c).
- An irregular shape spiculated mass lesion is seen at right retro areolar region, associated nipple retraction and increased peri-areolar skin thickness, by US it corresponds to right breast 12 o'clock retro-areolar region spiculated hypo echoic mass lesion with intra ductal extension reaching the nipple, such mass measures about 3.5 x 2 cm along its maximum dimensions.
- No left breast dominant masses, clustered microcalcifications or areas of architectural distortion seen.
- Normal left breast contour and skin thickening.
- Normal left nipple/areola complexes.
- The left breast parenchyma appears homogenous with no focal masses seen. Normal duct caliber.
- Multiple enlarged ipsilateral right axillary level 1 lymph nodes (>3) showing increased cortical thickness with eccentric fatty hilum with the largest measuring 1.9x1cm along its maximum dimensions. No supra or infra clavicular lymph nodes.
- Left axillary small nonspecific axillary lymph nodes with preserved fatty hilum.

Opinion :-

- **Right retro-areolar malignant looking mass lesion as described with ipsilateral pathological axillary level 1 lymph nodes (BIRADS 5), for histopathological correlation**
- **Left mammary fibroadenosis (BIRADS 2).**
- **CEUSM is recommended.**

Thanks for referral,
Dr. Eman Badawy, MD